

Started on	Sunday, 12 February 2023, 8:04 AM
State	Finished
Completed on	Sunday, 12 February 2023, 8:04 AM
Time taken	28 secs
Marks	0.00/88.00
Grade	0.00 out of 100.00



Question 1

Not answered

Marked out of 1.00

Which of the following is true concerning the use of antidepressants in eating disorders?

Select one:

- ☐ Medications should not be used as sole treatment for anorexia
- ☐ Antidepressants act slower in eating disorders than in depression
- ☐ Antidepressant effects are often sustained even after stopping the medication
- ☐ Antidepressants improve weight gain
- ☐ Antidepressants are more effective than CBT in anorexia

Your answer is incorrect.

Explanation:

Drugs have limited activity in anorexia nervosa, and none is currently licensed for this condition. Prompt restoration to a safe weight, family therapy, and structured psychotherapy are the main interventions. The aim of physical treatment is to improve nutritional health through re-feeding with very limited evidence to support the use of any pharmacological interventions other than those prescribed to correct metabolic deficiencies. Olanzapine is the only drug suggested to have any effect on weight restoration in anorexia nervosa. A Cochrane review found no evidence from four placebo-controlled trials that antidepressants improved weight gain, eating disorder or associated psychopathology. NICE has found little evidence to support the use of antidepressants. Antidepressants appear to have no role in anorexia nervosa.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 761-762.

The correct answer is: Medications should not be used as sole treatment for anorexia

Question 2

Not answered

Marked out of 1.00

With respect to the epidemiology of bulimia and anorexia, choose the incorrect statement.

Select one:

- ☐ The prevalence of bulimia is around 2%
- ☐ The stability of diagnoses of eating disorders is poor
- ☐ The prevalence of anorexia is around 0.5% to 1%
- ☐ Anorexia is excessively represented in lower social classes
- ☐ Bulimia generally starts later than anorexia

Your answer is incorrect.

Explanation:

Though it was once thought that anorexia nervosa (AN) was more common in higher social classes, this is not reflected in the current literature. AN can affect individuals of all social classes. The increase of AN in low-income and middle-income countries suggests that cultural traditions associated with industrialisation, urbanisation, and globalisation might be associated with environmental risk constellations for the development of AN. It is possible that western influences could increase the number of individuals who engage in behaviours such as strict dieting or excessive exercise, which can then trigger eating disorders in genetically susceptible individuals.

Epidemiology of Eating Disorders (ref NICE Guidelines 2017, Morris and Anderson 2020)
Prevalence of anorexia nervosa (AN) in Western countries: 1% females and < 0.5% males
Sex ratio in adults is 1:8 (men : women); less skewed in young people
AN is rare in children <13 years and relatively rare among middle-aged and elderly women
Peak age of onset of anorexia nervosa: 15-19 years
Anorexia nervosa has highest rate of mortality among all mental disorders; most common causes of death are suicide (20%) and cardiac complications
Bulimia nervosa (BN) is more prevalent than AN, with estimates of 1-4% in young women and lower in men; age of onset of BN is usually later than AN
Age at identification of BN is decreasing, currently at 15-24 years old
About half of those with BN have a history of AN, especially binge eating-purging type
Lifetime prevalence of binge-eating disorder (BED): approx 2% for women and 0.3% for men
Compared with other eating disorders, BED is more common in males and older people
BED is commonly associated with obesity and increased risk of mortality

SPMM COURSE



Ref: Zipfel S et al. Anorexia nervosa: aetiology, assessment, and treatment. The lancet psychiatry. 2015 Dec 1;2(12):1099-111.
 National Institute for Health and Clinical Excellence. NICE guideline Eating Disorders: recognition and treatment 2017.
 Morris J, Anderson S. An update on eating disorders. BJPsych Advances. 2020:1-11.
 The correct answer is: Anorexia is excessively represented in lower social classes



Question 3

Not answered

Marked out of 1.00

You are considering an antidepressant switch from phenelzine to fluoxetine for a 55-year-old lady with atypical depression. The minimum washout period required to enable the switching is

Select one:

- ☐ 48 hours
- ☐ 1 week
- ☐ 5 weeks
- ☐ 8 weeks
- ☐ 2 weeks

Your answer is incorrect.

Explanation:

When switching from phenelzine to fluoxetine, phenelzine should be tapered and stopped; then there should be a wait-time of 2 weeks before fluoxetine is started.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 338.

The correct answer is: 2 weeks

Question 4

Not answered

Marked out of 1.00

A 45-year-old man with major depression has been treated with a standard dose of fluoxetine for three months, and he has responded only partially. He suffers from severe insomnia and reports it as his most distressing symptom. Which one of the following drugs would you consider adding at this stage?

Select one:

- ☐ Trazodone
- ☐ Venlafaxine
- ☐ Duloxetine
- ☐ Mirtazapine
- ☐ Olanzapine

Your answer is incorrect.

Explanation:

Trazodone is commonly prescribed for insomnia (primary and secondary). It can not only improve insomnia in depressed patients treated with antidepressants but can also be an effective booster of antidepressant actions of other antidepressants. For secondary insomnia, once the underlying condition (e.g., depression) is in remission, trazodone treatment may be discontinued if insomnia does not re-emerge.

Ref: Stahl SM. Stahl's Essential Psychopharmacology: Prescriber's Guide, 7th Edition. 2021. p. 793.

The correct answer is: Trazodone

Question 5

Not answered

Marked out of 1.00

A 34-year-old man taking SSRIs develops erectile dysfunction. All of the following are appropriate management options except

Select one:

- ☐ Switch to a different class of antidepressants
- ☐ Reassure the patient that tolerance will develop in due course
- ☐ Reduce the dose of antidepressant prescribed
- ☐ Prescribe sildenafil citrate
- ☐ Switch to a different SSRI

Your answer is incorrect.

Explanation:

BAP guidelines note that sexual side effects can be **persistent** and may be especially relevant to those needing to take medication for the longer term. **Lowering the antidepressant dose** is a common clinical strategy.

Other strategies for **sexual side effects** including the following (BAP 2015):

- **Switching** to an antidepressant with a **lower tendency** to cause sexual side effects
- Adding **sildenafil or tadalafil** for erectile dysfunction, or **bupropion** 150mg twice daily for sexual dysfunction in men
- Adding **bupropion or sildenafil** in women

Ref: Cleare A et al. Evidence-based guidelines for treating depressive disorders with antidepressants: a revision of the 2008 British Association for Psychopharmacology guidelines. Journal of Psychopharmacology. 2015 May;29(5):459-525.

The correct answer is: Reassure the patient that tolerance will develop in due course

Question 6

Not answered

Marked out of 1.00

In ICD-11, unusual beliefs or magical thinking that influence the person's behaviour in ways that are inconsistent with subcultural norms are features of

Select one:

- ☐ Schizophrenia
- ☐ Bipolar disorder
- ☐ Schizoaffective disorder
- ☐ Schizotypal disorder
- ☐ Obsessive-compulsive disorder

Your answer is incorrect.

Explanation:

One feature of schizotypal disorder is unusual beliefs or magical thinking that influence the person's behaviour in ways that are inconsistent with subcultural norms, but do not reach the diagnostic requirements for a delusion. It should be noted that some people in the general population show eccentric behaviour and report psychotic-like or unusual subjective experiences without any apparent impairment in functioning. Schizotypal disorder should only be diagnosed if the individual is experiencing distress or impairment in personal, family, social, educational, occupational, or other important areas of functioning related to their symptoms.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Schizotypal disorder

Question 7

Not answered

Marked out of 1.00

Which of the following is NOT a characteristic feature of avoidant personality disorder?

Select one:

- ☐ Timidity
- ☐ Poor self esteem
- ☐ Rejection sensitivity
- ☐ Lack of confidence
- ☐ Preference for being alone

Your answer is incorrect.

Explanation:

People with avoidant personality disorder show extreme sensitivity to rejection and may lead socially withdrawn lives. Although shy, they are not asocial and show a great desire for companionship, but they need unusually strong guarantees of uncritical acceptance. Such people are characterised by:

- Avoidance of jobs with significant interpersonal contact for fear of criticism or rejection
- Unwillingness to get involved unless certain of being liked
- Restraint in relationships because of fear of being ridiculed
- Preoccupation with being criticized or rejected
- Social inhibition because of feelings of inadequacy
- Viewing themselves as inferior
- Reluctance to take risks because of possible embarrassment

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 560, 565.

American Psychiatric Association. Diagnostic and statistical manual of mental disorders (DSM-5®). American Psychiatric Pub; 2013.
The correct answer is: Preference for being alone



Question 8

Not answered

Marked out of 1.00

A poor prognostic factor for anorexia nervosa is

Select one:

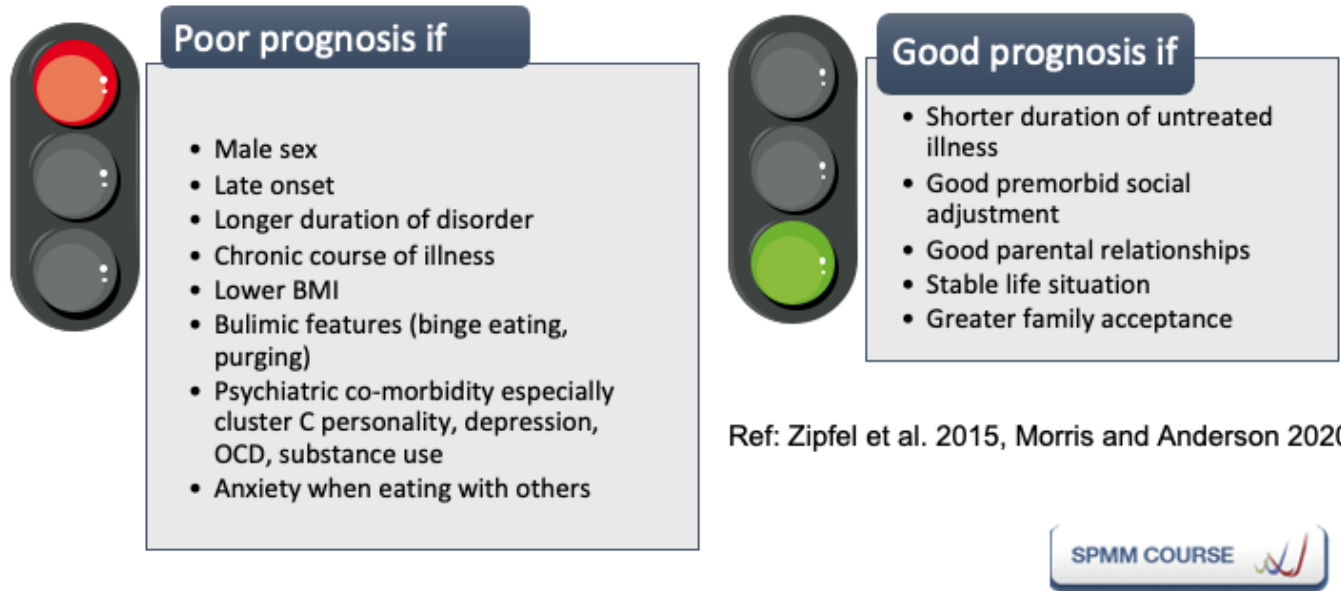
- ☐ Absence of purging behaviour
- ☐ Early age of onset
- ☐ Family acceptance
- ☐ Male sex
- ☐ Short duration of illness

Your answer is incorrect.

Explanation:



Prognostic factors of anorexia nervosa



Ref: Zipfel S et al. Anorexia nervosa: aetiology, assessment, and treatment. The lancet psychiatry. 2015 Dec 1;2(12):1099-111.
National Institute for Health and Clinical Excellence. NICE guideline Eating Disorders: recognition and treatment 2017.
Morris J, Anderson S. An update on eating disorders. BJPsych Advances. 2020:1-11.
The correct answer is: Male sex

Question 9

Not answered

Marked out of 1.00

A patient is grieving the death of one of his family members who died eight months ago. Your assessment indicates that he currently has symptoms of moderate depression, and a history of severe depression three years ago. What should be your next step?

Select one:

- ☐ Refer for bereavement counselling
- ☐ Prescribe an antidepressant and follow up
- ☐ Reassure and follow up
- ☐ Refer for psychodynamic psychotherapy
- ☐ Reassure and discharge

Your answer is incorrect.

Explanation:

As recommended by NICE, given that this patient currently has moderate depression as well as a past history of severe depression, an antidepressant should be considered.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the treatment and management of depression in adults. Updated edition 2019.

The correct answer is: Prescribe an antidepressant and follow up

Question 10

Not answered

Marked out of 1.00

A patient known to have bipolar disorder is currently on 500 mg valproate, 20 mg citalopram and 20 mg of olanzapine. He is presently irritable, tearful, and disinhibited. What is the first most appropriate thing to do?

Select one:

- ☐ Increase olanzapine
- ☐ Increase citalopram
- ☐ Stop citalopram
- ☐ Increase valproate
- ☐ Add lithium

Your answer is incorrect.

Explanation:

According to NICE and BAP guidelines, patients in a mixed state should be treated in the same way as those experiencing a manic episode. Antidepressants should be stopped/avoided if possible.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the assessment and management of bipolar disorder in adults, children and young people in primary and secondary care. Updated edition 2020.

Goodwin GM et al. Evidence-based guidelines for treating bipolar disorder: revised third edition recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2016 Jun;30(6):495-553.

The correct answer is: Stop citalopram

Question 11

Not answered

Marked out of 1.00

A 21-year-old man is brought to the psychiatric outpatient clinic by his father who is worried about the deterioration in his mental state. On examination, the patient is noted to have auditory hallucinations, and his father has noticed him talking to people who are not there. The patient also has a delusional belief that the devil has been tormenting him for many months. He is anxious and is progressively isolating himself from everyone. What is the lifetime prevalence for his probable condition (expressed in percentage)?

Select one:

- ☐ 0.05
- ☐ 10
- ☐ 20
- ☐ 3
- ☐ 1

Your answer is incorrect.

Explanation:

This man probably has schizophrenia.

Epidemiology of Psychosis and Schizophrenia (NICE Guidelines 2014; Jongsma et al., 2019)

Psychosis is relatively common (incidence 26.6/100,000 person-years), with schizophrenia the most common psychotic disorder

Schizophrenia - point prevalence 0.45% and lifetime expectancy 0.7%, though considerable variation in different areas and higher risk in urban environments often approximately 1% (lifetime prevalence)

Rates of schizophrenia generally reduce with age, though second peak in women starting mid-late 40s (unlike men, women display a bimodal age distribution; approx. 10% have onset > 40 years of age)

Men <45 yo have 2x rate of schizophrenia than women; no difference in incidence >45 yo;

Average age of onset in men = 23, in women = 26 (Oxford Handbook of Psychiatry, 2019)

Rates of schizophrenia higher in Black Caribbean and Black African migrants and descendants

¾ of those with schizophrenia experience recurrent relapse and some continued disability

Schizophrenia - within top 15 medical disorders causing disability

Mortality in schizophrenia is 2-3X above that of general population

Approx lifetime risk of suicide in schizophrenia = 5%

Increased risk of physical health problems with schizophrenia, including cardiovascular disease, obesity, metabolic aberrations and diabetes, as well as smoking and alcohol misuse

SPMM COURSE



Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the treatment and management of psychosis and schizophrenia in adults. Updated edition 2014.

Jongsma HE et al. International incidence of psychotic disorders, 2002–17: a systematic review and meta-analysis. The Lancet Public Health. 2019 May 1;4(5):e229-44.

The correct answer is: 1

Question 12

Not answered

Marked out of 1.00

A 19-year-old man with schizophrenia has been treated with an adequate dose of an atypical antipsychotic for 10 weeks without much improvement. There are no grounds to suspect treatment non-adherence. The least advisable next option for his treatment is

Select one:

- ☐ Exploring his drug and alcohol history
- ☐ Adding another antipsychotic
- ☐ Increasing the dose of his antipsychotic
- ☐ Switching to a different atypical antipsychotic
- ☐ Switching to a typical antipsychotic

Your answer is incorrect.

Explanation:

In the management of schizophrenia, antipsychotics should not be combined routinely, except briefly when switching from one medication to another.

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

The correct answer is: Adding another antipsychotic

Question 13

Not answered

Marked out of 1.00

A 35-year-old woman goes to her GP one month after the sudden death of her husband. She is tearful, not able to eat or sleep well, and has withdrawn from her friends while pining for her husband. She occasionally hears little whispers of her husband's voice at nighttime and she feels his presence. Her condition is

Select one:

- ☐ Stage two bereavement
- ☐ Severe depressive episode
- ☐ Stage three bereavement
- ☐ Stage one bereavement
- ☐ Delayed grief

Your answer is incorrect.

Explanation:

Grieving process models include at least three partially overlapping phases or states:

- 1) Initial shock, disbelief, and denial
- 2) Intermediate period of acute discomfort and social withdrawal
- 3) Culminating period of restitution and reorganisation

Phases of bereavement

Shock and denial (minutes, days, weeks)	Acute anguish (weeks, months)	Resolution (months, years)
• Disbelief • Numbness • Searching behaviour: pining, yearning, and protest	• Waves of somatic distress • Withdrawal • Preoccupation • Anger • Guilt Lost patterns of conduct • Restless and agitated • Aimless and amotivational • Identification with the bereaved	• Having grieved • Return to work • Resume old roles • Acquire new roles • Reexperience pleasure • Seek companionship and love of others

Adapted from Kaplan and Sadock's Synopsis of Psychiatry, 12th Ed, 2022. p. 852.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 852.

The correct answer is: Stage two bereavement

Question 14

Not answered

Marked out of 1.00

Mrs. Y is a 27-year-old pregnant woman with a history of episodic low mood and elated affect requiring treatment. She is currently euthymic. What is the risk of her relapse post-partum?

Select one:

- ☐ 33%
- ☐ 67%
- ☐ 4%
- ☐ 10%
- ☐ 20%

Your answer is incorrect.

Explanation:

Two-thirds of women with bipolar disorder will experience a relapse of illness post-partum. Risk is increased with a family history of post-partum psychosis, 4+ illness episodes pre-pregnancy, and (rapid) discontinuation of medication during pregnancy.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 493.

The correct answer is: 67%

Question 15

Not answered

Marked out of 1.00

A 32-year-old patient is hospitalised for the treatment of major depressive disorder. What is the likelihood that this patient will receive a subsequent diagnosis of bipolar disorder within the next 5 years?

Select one:

- ☐ 2-4%
- ☐ 40-50%
- ☐ 1-2%
- ☐ 7-12%
- ☐ 15-20%

Your answer is incorrect.

Explanation:

Estimated rates of conversion from unipolar to bipolar depression range from 0.37% per year to 3.95% per year. Predictably, the proportion of diagnostic conversions typically increases with the duration of follow-up: between 8.6 and 14% by 5 years, 17% by 10 years, and in certain studies as much as 45% by 15 years or 20% by 30 years. Most prospective studies have a follow-up period between 3 and 12 years. Angst's 2005 study had a >20-year follow-up. The rate of conversion from MDD to BD also increases with age, occurring in 2.2% from ages 10 to 14 as compared to 7% from ages 30-34. Among patients with depression, a family history of affective disorder, multiple depressive episodes, psychotic symptoms, treatment resistance, and early age of onset have been found to increase the risk for conversion to BD.

Ref: de la Vega D et al. A review on the general stability of mood disorder diagnoses along the lifetime. Current Psychiatry Reports. 2018 Apr;20(4):1-0.

Cegla-Schwartzman FB et al. Diagnostic Stability in Bipolar Disorder: A Narrative Review. Harvard review of psychiatry. 2019 Jan 1;27(1):3-14.

The correct answer is: 7-12%

Question 16

Not answered

Marked out of 1.00

ICD-11 does not endorse sub-typing schizophrenia using any symptom-based descriptive features. The subtypes of schizophrenia described in the previous version of ICD-10 were shown to have

Select one:

- ☐ Predictive validity
- ☐ Utility in treatment selection
- ☐ Longitudinal stability
- ☐ Lack of prognostic validity
- ☐ Lack of face validity

Your answer is incorrect.

Explanation:

In ICD-11, the nine ICD-10 schizophrenia subtypes (paranoid, hebephrenic, catatonic, etc.) were omitted because of their longitudinal instability, lack of prognostic validity, and lack of utility in treatment selection. They have been replaced by a system of coded symptom and course qualifiers.

Ref: Stein DJ et al. Mental, behavioral and neurodevelopmental disorders in the ICD-11: an international perspective on key changes and controversies. BMC medicine. 2020 Dec;18(1):1-24.

Reed GM et al. Innovations and changes in the ICD-11 classification of mental, behavioural and neurodevelopmental disorders. World Psychiatry. 2019 Feb;18(1):3-19.

The correct answer is: Lack of prognostic validity

Question 17

Not answered

Marked out of 1.00

The best intervention for acute lithium toxicity with neurological symptoms is

Select one:

- ☐ Activated charcoal
- ☐ IV potassium infusion
- ☐ IV sodium bicarbonate
- ☐ Haemodialysis
- ☐ IV frusemide

Your answer is incorrect.

Explanation:

Most patients experience toxic effects with lithium concentrations above 1.5 mmol/L. Concentrations above 2.0 mmol/L are associated with life-threatening toxicity and require urgent treatment. Haemodialysis may be needed to minimize toxicity.

The infographic is titled 'LITHIUM TOXICITY' in a vertical blue bar. It features three levels of toxicity, each in a box with a lightning bolt icon pointing to the next level. The first box (Mild toxicity) lists symptoms: Nausea/vomiting/diarrhea, Lethargy, Tremor, and Fatigue, with levels around 1.5 mEq/L. The second box (Moderate toxicity) lists symptoms: Confusion, Agitation, Delirium, Tachycardia, and Hypertonia, with levels up to 2.5 mEq/L. The third box (Severe toxicity) lists symptoms: Coma and seizures, Hyperthermia, Hypotension, Can be fatal, and May require haemodialysis (>2.5 mEq/L). To the left of the boxes is an illustration of a person sitting in a chair, looking unwell. To the right is an illustration of a yellow pill bottle and a glass of water. At the bottom right is a logo for 'SPMM COURSE'.

Mild toxicity:
Nausea/vomiting/diarrhea
Lethargy
Tremor
Fatigue
(levels ~1.5mEq/L)

Moderate toxicity:
Confusion
Agitation
Delirium
Tachycardia
Hypertonia
(up to 2.5 mEq/L)

Severe toxicity:
Coma and seizures
Hyperthermia
Hypotension
Can be fatal
May require haemodialysis
(>2.5 mEq/L)

LITHIUM TOXICITY

SPMM COURSE

Ref: Goodwin GM et al. Evidence-based guidelines for treating bipolar disorder: revised third edition recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2016 Jun;30(6):495-553.

The correct answer is: Haemodialysis

Question 18

Not answered

Marked out of 1.00

Mr. Y presents to the medical emergency department with a history of apathy, fatigue and weight loss. On further investigations, he has hyponatraemia and hyperkalemia. What is the most likely diagnosis?

Select one:

- ☐ Cushing syndrome
- ☐ Acute porphyria
- ☐ Addison disease
- ☐ Conn syndrome
- ☐ Diabetes insipidus

Your answer is incorrect.

Explanation:

Addison disease (primary adrenal insufficiency) is characterised by chronic glucocorticoid and/or mineralocorticoid deficiency due to failure of the adrenal cortex. It should be suspected in all acutely or chronically ill patients with general fatigue or severe weakness and unexplained dehydration, hypotension, weight loss, fever, abdominal pain, and hyperpigmentation. Key lab characteristics are hyponatremia, hyperkalemia, and (especially in children) hypoglycemia. Psychiatric symptoms include apathy, anhedonia, fatigue, and depressed mood.

Ref: Barthel A et al. An update on Addison's disease. Experimental and Clinical Endocrinology & Diabetes. 2019 Feb;127(02/03):165-75.

The correct answer is: Addison disease

Question 19

Not answered

Marked out of 1.00

Traumatic brain injury significantly increases the risk of which of the following?

Select one:

- ☐ Down syndrome
- ☐ Borderline personality disorder
- ☐ Asperger syndrome
- ☐ Psychosis
- ☐ Depression

Your answer is incorrect.

Explanation:

Depression is the most common psychiatric complication of traumatic brain injury (TBI), with prevalence rates of 30-40%. The aetiology of post-TBI depression is often difficult to determine because it may be driven by both psychosocial and biological factors. Some have proposed that TBI may cause psychosis, but other findings suggest that postinjury psychotic sequelae are relatively indistinguishable from idiopathic psychotic disorders. One study suggested that TBI may precipitate the development of schizophrenia in those with a genetic susceptibility, with a meta-analysis reporting a 60% increase in risk for schizophrenia after TBI. However, most of these findings are based on case reports or population-based studies using medical records to ascertain diagnoses. A 5-year prospective study following moderate to severe TBI found that all post-injury cases had a pre-injury history of psychosis. Findings of prospective studies have reported relatively low rates of psychotic disorders consistent with population rates.

Ref: Roy D et al. Risk factors for new-onset depression after first-time traumatic brain injury. *Psychosomatics*. 2018 Jan 1;59(1):47-57.

Ponsford J et al. Epidemiology and natural history of psychiatric disorders after TBI. *The Journal of neuropsychiatry and clinical neurosciences*. 2018 Oct;30(4):262-70.

The correct answer is: Depression

Question 20

Not answered

Marked out of 1.00

Which one of the following is a prominent risk factor for schizophrenia?

Select one:

- ☐ Lower social economic status
- ☐ Having a first-degree relative with schizophrenia
- ☐ Refugee status
- ☐ Older age
- ☐ Male sex

Your answer is incorrect.

Explanation:

Genetic factors account for the majority of liability to schizophrenia. Heritability estimates range from 60% to 80%. Estimates of schizophrenia liability can be based on the degree of relatedness to and number of affected relatives. It is likely that an individual needs to have several genes 'of small effect' that interact with each other and with time-specific exposure to other environmental risk factors.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 190-191.

The correct answer is: Having a first-degree relative with schizophrenia

Question **21**

Not answered

Marked out of 1.00

Mr. Y is a 40 year old gentleman with a long standing history of schizophrenia. Although he still harbours some chronic delusions of reference, these do not have any significant impact on his functioning. During his presentation to the outpatient clinic he is noted to be dishevelled, unwashed, and his affect could be described to be flat. He has no interest in any recreational activities and spends most of his time sitting in front of the television. Which of the following antipsychotic medications would be most appropriate to treat his condition?

Select one:

- ☐ Sertindole
- ☐ Olanzapine
- ☐ Haloperidol
- ☐ Perphenazine
- ☐ Chlorpromazine

Your answer is incorrect.

Explanation:

There are no well-replicated, large trials, or meta-analyses of trials, with negative symptoms as the primary outcome measure that have yielded convincing evidence for enduring and clinically significant benefit. Nevertheless, there are robust data suggesting superior efficacy against negative symptoms with certain antipsychotic treatment strategies, such as amisulpride and cariprazine, and that olanzapine and quetiapine may be more effective than risperidone. Augmentation with aripiprazole may also be effective.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 32-33.

The correct answer is: Olanzapine

Question **22**

Not answered

Marked out of 1.00

Tom is a 13-year-old boy who has been smoking cannabis over the last 6 months. His mum is concerned. She asks you by how much this will increase the chances of him developing schizophrenia. Your answer is

Select one:

- ☐ 2 fold increase
- ☐ 10 fold increase
- ☐ 8 fold increase
- ☐ No increase
- ☐ 4 fold increase

Your answer is incorrect.

Explanation:

Cannabis use, especially in adolescence, confers a **twofold increased risk** of later schizophrenia. The highest risk is for younger adolescents (**fourfold increased risk** for those using cannabis by **age 15**).

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the treatment and management of psychosis and schizophrenia in adults. Updated edition 2014.

Hollis C. Schizophrenia in children and adolescents. BJPsych Advances. 2015 Sep;21(5):333-41.

The correct answer is: 4 fold increase

Question **23**

Not answered

Marked out of 1.00

What would you do for a patient with OCD and no improvement after two months on 50mg of sertraline?

Select one:

- ☐ Switch to venlafaxine
- ☐ Add sodium valproate
- ☐ Change to a different SSRI
- ☐ Add an antipsychotic
- ☐ Increase the sertraline

Your answer is incorrect.

Explanation:

Doses of SSRIs licensed for the treatment of obsessive-compulsive disorder (OCD) are higher than those licensed for the treatment of depression. Initial response is usually slower to emerge than in depression (can take 10-12 weeks). Dose should be increased to gain maximal benefit.



Starting and target doses in OCD

Drug	Starting dose (mg/d)	Target dose (mg/d)
Fluoxetine	20	80
Fluvoxamine	50	300
Sertraline	50	200
Paroxetine	20	60
Citalopram	20	40
Escitalopram	10	40
Clomipramine	25	250

Adapted from Hirschtritt et al. 2017

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 424.

Hirschtritt ME, Bloch MH, Mathews CA. Obsessive-compulsive disorder: advances in diagnosis and treatment. Jama. 2017 Apr 4;317(13):1358-67.

The correct answer is: Increase the sertraline

Question **24**

Not answered

Marked out of 1.00

Which of the following is correct with respect to the prognosis of schizophrenia?

Select one:

- ☐ Hospitalisation is reduced in smokers
- ☐ Poor premorbid functioning increases early suicide rates
- ☐ Female patients have a higher rate of suicide
- ☐ Presence of mood symptoms has no influence on outcome
- ☐ Prognosis is better in developing countries

Your answer is incorrect.

Explanation:

Positive prognostic factors in schizophrenia include acute onset, female sex, living in a developing country, marked mood disturbance (especially elation) during initial presentation, and family history of affective disorder.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 350.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 207.

The correct answer is: Prognosis is better in developing countries

Question 25

Not answered

Marked out of 1.00

When taken as needed, beta blockers such as atenolol can reduce performance anxiety in social anxiety disorder. How long before an anticipated performance should atenolol be taken in order to be effective?

Select one:

- ☐ 20 minutes
- ☐ 4 hours
- ☐ 8 hours
- ☐ 1 day
- ☐ 1 hour

Your answer is incorrect.

Explanation:

Beta blockers have been validated for the treatment of performance-only social anxiety, which generally arises in the context of public speaking or other performative situations. The non-selective beta blocker propranolol and the cardio-selective agent atenolol are the two primary agents that have been studied for use in performance-only social anxiety disorder. Recommended use is as needed 1-2 hours before a performance situation at total daily doses of 10-80mg propranolol and 50-150mg atenolol.

Ref: Melaragno AJ. Pharmacotherapy for Anxiety Disorders: From First-Line Options to Treatment Resistance. Focus. 2021 Jun;19(2):145-60.

The correct answer is: 1 hour

Question **26**

Not answered

Marked out of 1.00

A 34-year-old man with insomnia is interested in short-term treatment with hypnotics. His most prominent problem is frequent awakenings through the course of sleep. He is able fall asleep normally. Which of the following agents is the best choice?

Select one:

- ☐ Zolpidem
- ☐ Zopiclone
- ☐ Temazepam
- ☐ Midazolam
- ☐ Zaleplon

Your answer is incorrect.

Explanation:

Temazepam is commonly prescribed for the short-term treatment of insomnia. It is notable for its delayed onset of action compared to some other sedative hypnotics. Temazepam has slow gastrointestinal absorption compared to other sedatives, so may be more effective for nocturnal awakening than for initial insomnia (unless dosed 1-2 hours prior to bedtime).

Ref: Stahl SM. Stahl's Essential Psychopharmacology: Prescriber's Guide, 7th Edition. 2021. pp. 755-758.

The correct answer is: Temazepam

Question **27**

Not answered

Marked out of 1.00

A 34-year-old man complains of experiencing ejaculation within a few seconds of initiating sexual acts. He is very embarrassed, but is seeking treatment. Which of the following is the best treatment option?

Select one:

- ☐ Tricyclics
- ☐ Bupropion
- ☐ SSRIs
- ☐ Phospho-diesterase inhibitors
- ☐ Benzodiazepines

Your answer is incorrect.

Explanation:

The pathophysiology of premature ejaculation is relatively unknown. One aetiological possibility is dysfunction of 5-HT receptors. It is believed that SSRIs induce delayed ejaculation by inhibiting presynaptic and somatodendritic serotonin transporters, thereby increasing total central 5-HT neurotransmission and activation of postsynaptic 5-HT receptors. The effect of SSRIs on premature ejaculation can be seen as early as a few days after starting the medication. However, most patients notice a significant increase in ejaculatory time 1-2 weeks after beginning the medication.

Ref: Martin C et al. Current and emerging therapies in premature ejaculation: Where we are coming from, where we are going. International Journal of Urology. 2017 Jan;24(1):40-50.

The correct answer is: SSRIs

Question 28

Not answered

Marked out of 1.00

Which of the following drugs is most likely to cause hyponatraemia?

Select one:

- ☐ Citalopram
- ☐ Lofepramine
- ☐ Risperidone
- ☐ Lithium
- ☐ Duloxetine

Your answer is incorrect.

Explanation:

Hyponatremia is most likely to occur in association with SSRIs, including citalopram, and anticonvulsants, particularly carbamazepine and oxcarbazepine. The estimates of incidence vary as per definitions used with 0.06 to 40% in some studies for SSRIs. Among SSRIs, fluoxetine, citalopram and escitalopram appear to have higher incidence rates (.078% to .085%) than paroxetine and sertraline (.033% to .053%). Pharmacoepidemiological studies indicate that citalopram is a common offender.

Ref: Butterfield DJ, Eaves S, Ott C. Psychotropic-induced hyponatremia. Current Psychiatry. 2019 Feb 1;18(2):36-42.

Shepshelovich D, et al. Medication-induced SIADH: distribution and characterization according to medication class. British journal of clinical pharmacology. 2017 Aug;83(8):1801-7.

The correct answer is: Citalopram

Question 29

Not answered

Marked out of 1.00

Choose an accurate statement with regard to depression in patients with a terminal illness.

Select one:

- ☐ It does not respond to antidepressants
- ☐ It is generally diagnosed easily
- ☐ It may respond to amphetamines
- ☐ Depression is uncommon in palliative care settings
- ☐ Somatic symptoms are the most useful in diagnosis

Your answer is incorrect.

Explanation:

Depression is commonly encountered in palliative care patients. Depressive symptoms are present in 15-50% of patients with cancer and 5-20% will meet criteria for MDD. Dying patients may underreport or consciously disguise depressive symptoms in their final weeks of life. Neurovegetative symptoms of depression (anergia, fatigue, weight loss, anorexia, impaired concentration, sleep disturbance) are also common manifestations of advanced medical illness. Also, clinicians may erroneously believe that depression is an appropriate response to the dying process. Somatic symptoms are not useful when diagnosing depression in such cases; pervasive global anhedonia is a more useful criterion for the diagnosis of depression. Treatment of depression includes SSRIs, SNRIs, low dose amitriptyline (avoid in those with higher risk of delirium; useful if neuropathic pain is present), and lofepramine. Rapid-acting psychostimulants (e.g. dexamphetamine, methylphenidate, methylamphetamine, and pemoline) may be useful in patients with a short life expectancy.

Ref: Aziz VM, Saeed R. Palliative care for older people: the psychiatrist's role. BJPsych Advances. 2019 Jan;25(1):37-46.

The correct answer is: It may respond to amphetamines

Question 30

Not answered

Marked out of 1.00

In which of the following anxiety disorders is a beta blocker particularly useful?

Select one:

- ☐ Generalised anxiety disorder
- ☐ Animal phobia
- ☐ Blood injury injection phobia
- ☐ Performance-only social anxiety disorder
- ☐ Generalised social anxiety disorder

Your answer is incorrect.

Explanation:

Beta blockers have been validated for the treatment of performance-only social anxiety disorder, which generally arises in the context of public speaking or other performative situations. Their hypothesised mechanism of action is suppression of the physiological hyperarousal that occurs in the fear response to such situations. This process interrupts an escalating feedback loop in which patients become distressed and self-conscious about physical manifestations such as flushing, palpitations, and sweating, which subsequently causes them to further amplify these symptoms. This amplification, in turn, begets more psychic anxiety about these manifestations being scrutinised by others and so on. For reasons that are not fully understood, beta blockers have not demonstrated efficacy in treating generalised social anxiety disorder.

Ref: Melaragno AJ. Pharmacotherapy for Anxiety Disorders: From First-Line Options to Treatment Resistance. Focus. 2021 Jun;19(2):145-60.

The correct answer is: Performance-only social anxiety disorder

Question **31**

Not answered

Marked out of 1.00

A 45-year-old lady with bipolar affective disorder develops sores in her mouth. Which of the following drugs is most likely to be implicated?

Select one:

- ☐ Clonazepam
- ☐ Haloperidol
- ☐ Lamotrigine
- ☐ Olanzapine
- ☐ Lithium

Your answer is incorrect.

Explanation:

Lamotrigine can cause ulcers in the mouth, throat, nose or, genitals. It can also cause skin rashes or redness, which may develop into life-threatening skin reactions including widespread rash with blisters and peeling skin, particularly occurring around the mouth, nose, eyes, and genitals (Stevens-Johnson syndrome), extensive peeling of the skin (more than 30% of the body surface – toxic epidermal necrolysis), or extended rashes with liver, blood and other body organs involvement (drug reaction with eosinophilia and systemic symptoms or DRSS hypersensitivity syndrome).

Ref: <https://www.medicines.org.uk/emc/files/pil.8052.pdf>

The correct answer is: Lamotrigine

Question **32**

Not answered

Marked out of 1.00

What is the most characteristic feature expected in a patient with serotonin syndrome?

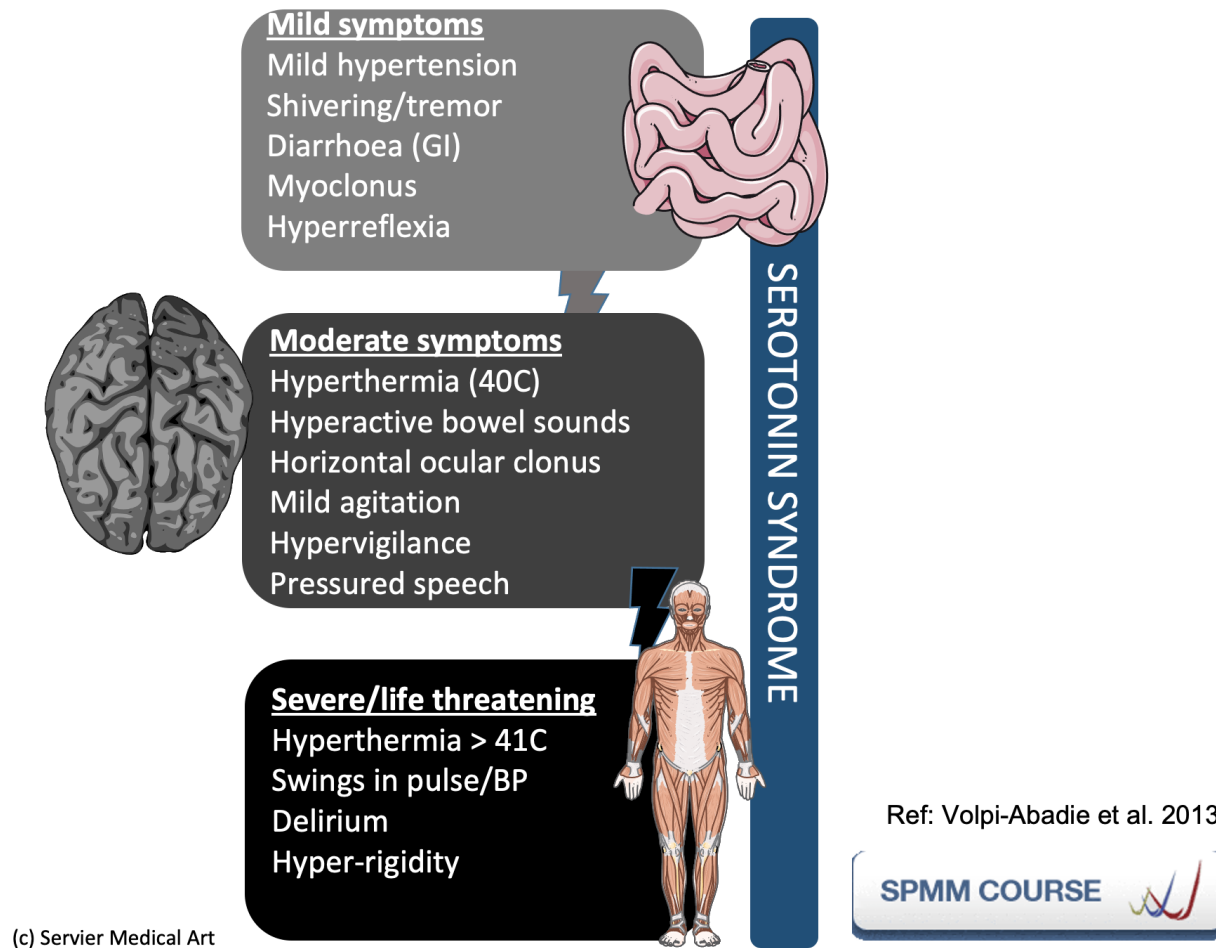
Select one:

- ☐ Hyperreflexia
- ☐ Elevated white cells
- ☐ Blurred vision
- ☐ Confusion
- ☐ Akathisia

Your answer is incorrect.

Explanation:

Tremor, clonus, and agitation in the absence of extrapyramidal signs should cause the clinician to consider serotonin syndrome. The diagnosis can be complicated by the fact that rigidity can occur and mask clonus and hyperreflexia. In contrast to neuroleptic malignant syndrome, serotonin syndrome can present with hyperreflexia and mydriasis (dilated pupils), and on blood testing white cell counts and creatinine kinase levels are typically within the normal range.



Ref: Taylor DM, Gaughran F, Pillinger T. The Maudsley Practice Guidelines for Physical Health Conditions in Psychiatry. 2021. pp. 683-684.

Volpi-Abadie J et al. Serotonin syndrome. Ochsner Journal. 2013 Dec 21;13(4):533-40.

The correct answer is: Hyperreflexia

Question 33

Not answered

Marked out of 1.00

According to the description in ICD-11, patients with schizotypal disorder must have

Select one:

- ☐ First rank symptoms
- ☐ Delusions of reference
- ☐ Duration of symptoms lasting at least several years
- ☐ Bizarre delusions
- ☐ Passivity phenomenon

Your answer is incorrect.

Explanation:

According to ICD-11, schizotypal disorder is characterised by:

- An enduring pattern (i.e. characteristic of the person's functioning over a period of at least several years)
- Eccentricities in behaviour, appearance and speech
- Cognitive and perceptual distortions, unusual beliefs, and discomfort with interpersonal relationships
- May include constricted or inappropriate affect and anhedonia
- May include paranoid ideas, ideas of reference, or other psychotic symptoms, but are not of sufficient intensity or duration to meet the diagnostic requirements of schizophrenia, schizoaffective disorder, or delusional disorder
- Symptoms cause distress/impairment in personal, family, social, educational, occupational or other areas of functioning

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed,; ICD-11; World Health Organization, 2020)

The correct answer is: Duration of symptoms lasting at least several years

Question **34**

Not answered

Marked out of 1.00

Compared to the general population, people with schizophrenia die approximately how many years earlier?

Select one:

- ☐ 1-2 years
- ☐ 25-30 years
- ☐ 10-20 years
- ☐ 3-5 years
- ☐ 5-8 years

Your answer is incorrect.

Explanation:

Those with schizophrenia have 2-3X mortality risk compared to the general population - a mortality gap of 6-25 years for schizophrenia versus the general population. As per NICE, males die approximately 20 years earlier and females die 15 years earlier than general population. About 1/3 of premature deaths are from suicide and accidents, but most are accounted for by physical disorders. Major natural causes of death in schizophrenia include cardiovascular (e.g. coronary heart disease), cerebrovascular (e.g. stroke), respiratory (e.g. COPD), metabolic (e.g. diabetes), and infectious (e.g. HIV, hepatitis C) disease, as well as cancer. Long-term treatment is associated with a lower mortality compared with no antipsychotic use. Relevant risk factors include cigarette smoking, diet, exercise, obesity, relative poverty, and poor healthcare. Under-recognition and under-treatment of physical disorders in patients with schizophrenia contributes to poorer outcomes compared with the general population. Suicide rates are higher in schizophrenia than the general population, and highest for those not treated with antipsychotics.

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

The correct answer is: 10-20 years

Question **35**

Not answered

Marked out of 1.00

Which one among the following statements is incorrect about schizophrenia?

Select one:

- ☐ 10% of women with schizophrenia are first diagnosed after 40 years of age
- ☐ Schizophrenia is more common in immigrants
- ☐ Schizophrenia is more common in males than females throughout the lifespan
- ☐ Schizophrenia is more common in urban areas and in lower social economic classes
- ☐ Males are diagnosed about 5 years younger than females

Your answer is incorrect.

Explanation:



Epidemiology of Psychosis and Schizophrenia (NICE Guidelines 2014; Jongsma et al., 2019)

Psychosis is relatively common (incidence 26.6/100,000 person-years), with schizophrenia the most common psychotic disorder

Schizophrenia - point prevalence 0.45% and lifetime expectancy 0.7%, though considerable variation in different areas and higher risk in urban environments often approximately 1% (lifetime prevalence)

Rates of schizophrenia generally reduce with age, though second peak in women starting mid-late 40s (unlike men, women display a bimodal age distribution; approx. 10% have onset > 40 years of age)

Men <45 yo have 2x rate of schizophrenia than women; no difference in incidence >45 yo;

Average age of onset in men = 23, in women = 26 (Oxford Handbook of Psychiatry, 2019)

Rates of schizophrenia higher in Black Caribbean and Black African migrants and descendants

¾ of those with schizophrenia experience recurrent relapse and some continued disability

Schizophrenia - within top 15 medical disorders causing disability

Mortality in schizophrenia is 2-3X above that of general population

Approx lifetime risk of suicide in schizophrenia = 5%

Increased risk of physical health problems with schizophrenia, including cardiovascular disease, obesity, metabolic aberrations and diabetes, as well as smoking and alcohol misuse

SPMM COURSE



Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the treatment and management of psychosis and schizophrenia in adults. Updated edition 2014.

Jongsma HE et al. International incidence of psychotic disorders, 2002–17: a systematic review and meta-analysis. The Lancet Public Health. 2019 May 1;4(5):e229-44.

The correct answer is: Schizophrenia is more common in males than females throughout the lifespan

Question **36**

Not answered

Marked out of 1.00

A young man started on risperidone presents with sexual side effects. Which investigation would you perform now?

Select one:

- ☐ Testosterone levels
- ☐ Fasting glucose level
- ☐ Thyroid function
- ☐ Prolactin levels
- ☐ Androgen levels

Your answer is incorrect.

Explanation:

BAP guidelines (2020) note the following:

- Plasma prolactin elevation is linked to D2/3 blockade in the pituitary gland
- Risk of plasma prolactin elevation is greatest with antipsychotics such as risperidone, paliperidone, amisulpride, and haloperidol and seems to be lowest with medications such as aripiprazole, clozapine, brexpiprazole, cariprazine, and quetiapine
- Plasma prolactin elevation is asymptomatic in some patients, while others may exhibit side effects due to the direct effects of prolactin on body tissues (such as galactorrhea) and secondary side effects including endocrine-related sexual and reproductive dysfunction

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

The correct answer is: Prolactin levels

Question 37

Not answered

Marked out of 1.00

A 34-year-old inpatient is on clozapine therapy for treatment resistant schizophrenia. He has weekly blood count monitoring. In which of the following situations is caution, with further sampling, advised (amber light)?

Select one:

- ☐ WCC levels of 300cells/cu
- ☐ WCC levels of 9200cells/cu
- ☐ WCC levels of 250cells/cu
- ☐ WCC levels of 3000cells/cu
- ☐ WCC levels of 9000 cells/cu

Your answer is incorrect.

Explanation:

When an 'amber light' is issued, caution – with further sampling – is advised. This occurs when either the white cell count (WCC) falls to 3000-3500/mm³ or the absolute neutrophil counts falls to 1500-2000/mm³. Blood monitoring must be performed at least twice weekly until the WCC and absolute neutrophil count stabilise within the range of 3000-3500/mm³ and 1500-2000/mm³, respectively, or higher.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 221.

The correct answer is: WCC levels of 3000cells/cu

Question **38**

Not answered

Marked out of 1.00

Which of the following medications is least affected by the variations in CYP2D6 mediated metabolism?

Select one:

- ☐ Paroxetine
- ☐ Fluoxetine
- ☐ Clomipramine
- ☐ Venlafaxine
- ☐ Reboxetine

Your answer is incorrect.

Explanation:

Reboxetine is mostly metabolised by CYP3A4 metabolism. Substrates of CYP2D6 include clomipramine, fluoxetine, paroxetine, and venlafaxine.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 854-855.

The correct answer is: Reboxetine

Question 39

Not answered

Marked out of 1.00

What is the mechanism behind QT prolongation caused by antipsychotics?

Select one:

- ☐ Blocking cardiac potassium channels
- ☐ Torsades de pointes
- ☐ Increasing number of fast acting sodium channels
- ☐ Blocking I type calcium channels
- ☐ Stimulation of vagal nerve

Your answer is incorrect.

Explanation:

Some antipsychotics block cardiac potassium channels and are linked to prolongation of the cardiac QT interval, a risk factor for the ventricular arrhythmia torsades de pointes, which is sometimes fatal.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 141.

The correct answer is: Blocking cardiac potassium channels

Question 40

Not answered

Marked out of 1.00

A patient has a history of pervasive distrust and suspiciousness regarding his neighbours. Which of the following features will strongly favour a diagnosis of schizophrenia instead of paranoid personality disorder?

Select one:

- ☐ Very early age of onset
- ☐ History of depressive episodes
- ☐ Presence of positive psychotic symptoms
- ☐ Family history of schizophrenia
- ☐ Continuous, non episodic nature of symptoms

Your answer is incorrect.

Explanation:

Paranoid personality disorder can be differentiated from schizophrenia and other primary psychotic disorders by the absence of fixed delusions, hallucinations, and formal thought disorder. People with paranoid personality disorder have longstanding, non-episodic suspiciousness and mistrust, and are characterised by:

- Suspiciousness that others are exploiting, harming, or deceiving them
- Reading hidden demeaning or threatening meanings into benign remarks/events
- Holding longstanding grudges
- Believing that others are not loyal or trustworthy
- Reluctance to confide in others
- Feeling that others are attacking their character or reputation
- Recurrent suspicions of infidelity

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 562.

American Psychiatric Association. Diagnostic and statistical manual of mental disorders (DSM-5®). American Psychiatric Pub; 2013 May 22.
The correct answer is: Presence of positive psychotic symptoms



Question 41

Not answered

Marked out of 1.00

Which one of the following is **not** an environmental factor known to be associated with an increased risk of schizophrenia?

Select one:

- ☐ Maternal influenza in pregnancy and winter births
- ☐ Delayed walking and neurodevelopmental difficulties
- ☐ Degree of urbanisation at birth
- ☐ Complications of pregnancy, delivery and neonatal period
- ☐ Positive family history of schizophrenia

Your answer is incorrect.

Explanation:

Positive family history is a genetic factor and not an environmental factor associated with an increased risk of schizophrenia. Genetic factors account for the majority of liability to schizophrenia and heritability estimates range from 60% to 80%.

Environmental factors associated with an increased risk of schizophrenia:

- Complications of pregnancy, delivery, and the neonatal period
- Delayed walking and neurodevelopmental difficulties
- Early social services contact and disturbed childhood behaviour
- Severe maternal malnutrition
- Maternal influenza in pregnancy and winter births
- Degree of urbanisation at birth
- Use of cannabis, especially during adolescence

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 190-191.

The correct answer is: Positive family history of schizophrenia

Question **42**

Not answered

Marked out of 1.00

Which of the following classes of drugs can cause peripheral neuropathy as a rare side effect?

Select one:

- ☐ MAOIs
- ☐ SSRIs
- ☐ SNRIs
- ☐ TCAs
- ☐ NARI

Your answer is incorrect.

Peripheral neuropathy has been reported as a rare side effect of MAOIs like phenelzine, isocarboxazid and tranylcypromine.

The correct answer is: MAOIs

Question 43

Not answered

Marked out of 1.00

A 33-year-old man has longstanding odd eccentric behaviour, social withdrawal and magical thinking with intense illusions. His most likely diagnosis is

Select one:

- ☐ Asperger syndrome
- ☐ Schizotypal disorder
- ☐ Schizoaffective disorder
- ☐ Bipolar disorder
- ☐ Schizophrenia

Your answer is incorrect.

Explanations:

According to ICD-11, schizotypal disorder is characterised by eccentricities in behaviour, appearance and speech, as well as cognitive and perceptual distortions, unusual beliefs, and discomfort with— and often reduced capacity for— interpersonal relationships. It may include paranoid ideas, ideas of reference, or other psychotic symptoms, including hallucinations in any modality, but are not of sufficient intensity or duration to meet the diagnostic requirements of schizophrenia, schizoaffective disorder, or delusional disorder. Another feature of schizotypal disorder is unusual beliefs or magical thinking that influence the person's behaviour in ways that are inconsistent with subcultural norms, but do not reach the diagnostic requirements for a delusion.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed.; ICD-11; World Health Organization, 2020)

The correct answer is: Schizotypal disorder

Question **44**

Not answered

Marked out of 1.00

The lifetime prevalence of obsessive-compulsive personality disorder is

Select one:

- ☐ 2-3%
- ☐ 1-2%
- ☐ 5-10%
- ☐ 10-15%
- ☐ 4-5%

Your answer is incorrect.

Explanation:



Epidemiology of OCD

Features	Specifics
Lifetime prevalence	Fairly constant – around 2-3%; fourth most commonly prevalent psychiatric disorder in epidemiological studies (after phobias, alcohol use disorder, and depression)
Point prevalence	1-3% of adults and 1-2% of children and adolescents (community samples)
Gender differences	Gender ratio of OCD in clinical samples is roughly equal, but females predominate in community populations (~1.5:1); males have a trend toward more tics and poorer outcome than females
Age of onset	Mean age of onset is bimodal, with peaks at 11 years and 23 years (2/3 rd before 25 years); early-onset OCD is more common in males



Ref: National Institute for Health and Clinical Excellence. NICE guideline Obsessive-compulsive disorder: core interventions in the treatment of obsessive-compulsive disorder and body dysmorphic disorder. Updated edition 2020.

The correct answer is: 2-3%

Question 45

Not answered

Marked out of 1.00

Which of the following statements is an accurate description of the evidence for olanzapine as a treatment for schizophrenia?

Select one:

- ☐ Pragmatic trials show lower discontinuation rates for olanzapine because of weight gain than typical antipsychotics
- ☐ Olanzapine is better than clozapine at reducing suicide attempts in schizophrenia patients
- ☐ Olanzapine is more effective than typical antipsychotics at reducing psychotic symptoms
- ☐ Olanzapine is associated with fewer metabolic effects than first-generation antipsychotics
- ☐ Olanzapine is associated with fewer extrapyramidal adverse effects than typical antipsychotic drugs

Your answer is incorrect.

Explanation:

CATIE (Clinical Antipsychotic Trials of Intervention Effectiveness) and CUtLASS (Cost Utility of the Latest Antipsychotic Drugs in Schizophrenia Study) were both pragmatic clinical trials with no sponsorship by industry. Both trials had broad inclusion criteria and long follow-up, and tried to mimic clinical routine. In CATIE, patients assigned to olanzapine had the longest time to discontinuation as a result of inadequate efficacy, the longest successful treatment time and the fewest hospitalisations as a result of exacerbation of schizophrenia. Olanzapine was associated with more discontinuation for weight gain or metabolic effects, and perphenazine with more discontinuation for EPS. Overall, the authors of both trials concluded that second-generation antipsychotics (SGAs) do not markedly differ from FGAs regarding compliance, quality of life, and effectiveness.

Two non-regulatory open-label trials, conducted in 'real-world conditions' similar to CATIE and CUtLASS, are the observational, non-randomised European SOHO (Schizophrenia Outpatient Health Outcomes) study and the randomised EUFEST (European First-Episode Schizophrenia Trial). Both studies investigated a less chronically ill population and had much higher retention rates compared with CATIE and CUtLASS, and found some relevant advantages of SGAs over FGAs. In EUFEST, there were significant difference in the Clinical Global Impression (CGI) and the Global Assessment of Functioning (GAF) scores, with greatest improvements seen with amisulpride and olanzapine, and least improvements with quetiapine and haloperidol. Moreover, relevant differences were found favouring SGAs with regard to retention rate (olanzapine with the highest) and motor adverse effects.

InterSePT (International Suicide Prevention Trial), a prospective RCT, showed a 26% reduction in the risk of suicide attempts in clozapine-treated patients compared to those using olanzapine.

Ref: Naber D, Lambert M. The CATIE and CUtLASS studies in schizophrenia. CNS drugs. 2009 Aug 1;23(8):649-59.

Swartz MS et al. Special section on implications of CATIE: what CATIE found: results from the schizophrenia trial. Psychiatric services. 2008 May;59(5):500-6.

Turecki G et al. Suicide and suicide risk. Nature reviews Disease primers. 2019 Oct 24;5(1):1-22.

The correct answer is: Olanzapine is associated with fewer extrapyramidal adverse effects than typical antipsychotic drugs



Question **46**

Not answered

Marked out of 1.00

A 45-year-old man with bipolar disorder has been taking lithium for many years. He has now developed renal impairment. The medication of choice would be

Select one:

- ☐ Chlorpromazine
- ☐ Clonazepam
- ☐ Haloperidol
- ☐ Diazepam
- ☐ Sodium valproate

Your answer is incorrect.

Explanation:

According to NICE guidelines, lithium is the most effective long-term treatment for bipolar disorder. However, if lithium is poorly tolerated or not suitable (for example, in the setting of renal impairment), either valproate or olanzapine should be used instead. Quetiapine may be considered if quetiapine was effective during a previous episode of mania or depression.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the assessment and management of bipolar disorder in adults, children and young people in primary and secondary care. Updated edition 2020.

The correct answer is: Sodium valproate

Question **47**

Not answered

Marked out of 1.00

Sleep terrors are associated with disturbance in which of the following stages of sleep?

Select one:

- ☐ Superficial sleep
- ☐ REM sleep
- ☐ Slow wave sleep
- ☐ Stage 2 sleep
- ☐ Stage 1 sleep

Your answer is incorrect.

Explanation:

Sleep terrors are recurrent episodes of abrupt awakening from sleep characterised by a panicky scream, with intense fear and autonomic arousal. The individual usually has no recall of the details of the event and is unresponsive to stimuli (e.g conversation) during the episode. Like sleepwalking, these episodes usually arise from slow-wave sleep. Unlike nightmares, in which an elaborate dream sequence unfolds, sleep terrors may be devoid of images or contain only fragments of very brief but frighteningly vivid and sometimes static images. Psychopathology is seldom associated with sleep terrors in children; however, a history of a traumatic experience or frank psychiatric problems is often co-morbid in adults with this disorder.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 499.

The correct answer is: Slow wave sleep

Question 48

Not answered

Marked out of 1.00

Which one of the following findings has not been demonstrated in patients diagnosed with schizophrenia?

Select one:

- ☐ Reduced volume in the prefrontal cortex
- ☐ Decreased prefrontal white matter
- ☐ Decreased prefrontal grey matter
- ☐ Reduced ventricular size
- ☐ Reduction in the size of medial temporal lobe structures

Your answer is incorrect.

Explanation:

Neuropathology in schizophrenia includes:

- Larger ventricles
- Decreases in gray matter and whole-brain volume
- White matter anisotropy
- Reduced prefrontal lobe volume
- Reduced volume of medial temporal lobe

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 358-359.

Hollis C. Schizophrenia in children and adolescents. BJPsych Advances. 2015 Sep;21(5):333-41.

Bakhshi K, Chance SA. The neuropathology of schizophrenia: a selective review of past studies and emerging themes in brain structure and cytoarchitecture. Neuroscience. 2015 Sep 10;303:82-102.

The correct answer is: Reduced ventricular size



Question 49

Not answered

Marked out of 1.00

The most common neurological side effect seen when using fluoxetine is

Select one:

- ☐ Headache
- ☐ Peripheral neuropathy
- ☐ Tremor
- ☐ Seizure
- ☐ Parkinsonism

Your answer is incorrect.

Explanation:

The incidence of headache in SSRI trials was 18-20%, only 1% higher than the placebo rate. Fluoxetine is the most likely to cause headaches. Conversely, SSRIs have been used as prophylaxis against both migraine and tension-type headaches, though the data is mixed.

Of note, tremor is also a common side effect of SSRIs and SNRIs. Fluoxetine has been frequently associated with a 6-12 Hz frequency, postural tremor and observed with a dose of 26mg per day. Some people with well-controlled Parkinson disease may experience acute worsening of their motor symptoms when they take SSRIs.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 629.

Revet A et al. Antidepressants and movement disorders: a postmarketing study in the world pharmacovigilance database. BMC psychiatry. 2020 Dec;20(1):1-3.

Baizabal-Carvalho JF, Morgan JC. Drug-induced tremor, clinical features, diagnostic approach and management. Journal of the Neurological Sciences. 2022 Apr 15;435:120192.

The correct answer is: Headache

Question **50**

Not answered

Marked out of 1.00

The most likely good prognostic factor in schizophrenia for a patient of age 25 is

Select one:

- ☐ Lack of precipitating factors
- ☐ Female gender
- ☐ Social withdrawal
- ☐ Insidious onset
- ☐ Being a smoker

Your answer is incorrect.

Explanation:

Positive prognostic factors in schizophrenia include acute onset, female sex, living in a developing country, marked mood disturbance (especially elation) during initial presentation, and family history of affective disorder.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 350.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 207.

The correct answer is: Female gender

Question **51**

Not answered

Marked out of 1.00

Of the following, the strongest predictor of mortality in a patient with anorexia nervosa is

Select one:

- ☐ Alcohol use
- ☐ Cannabis use
- ☐ History of panic attacks
- ☐ Younger age of onset
- ☐ Smoking

Your answer is incorrect.

Explanation:

Comorbid psychiatric disorders, as well as BMI at hospitalisation and older age at onset, have been identified as risk factors for poorer long-term outcomes in AN, including mortality. Specifically, substance use disorders, self-harm, and affective disorders are considered predictors of mortality. Substance abuse, especially alcohol use disorder, increases mortality from natural causes of death in patients with AN.

Ref: Kask J et al. Mortality in women with anorexia nervosa: the role of comorbid psychiatric disorders. Psychosomatic medicine. 2016 Oct 1;78(8):910-9.

The correct answer is: Alcohol use

Question 52

Not answered

Marked out of 1.00

Which of the following statements about remission and recovery from first episode psychosis (FEP) is correct?

Select one:

- ☐ Approximately 25% of FEP patients achieve remission
- ☐ Approximately 15% of FEP patients achieve recovery
- ☐ Recovery rates stabilise after the first 2 years of illness
- ☐ Lower remission rates have been seen in more recent studies
- ☐ The long-term prognosis for FEP has been worsening with time

Your answer is incorrect.

Explanation:

'Remission' from first-episode psychosis (FEP) may be defined as mild or absent symptoms for at least 6 months. 58% of patients with FEP achieve remission (over a mean follow-up period of 5.5 years). Higher remission rates have been seen in studies from more recent years compared to older studies. 'Recovery' refers to sustained symptomatic and functional improvement for more than 2 years. 38% of patients with FEP achieve recovery (over a mean follow-up period of 7.2 years). Recovery rates stabilise after the first 2 years of illness, suggesting that psychosis is not a progressively deteriorating illness state. There is now a better long-term prognosis in FEP and first-episode schizophrenia, and a more positive outlook for people diagnosed with these conditions.

Ref: Lally J et al. Remission and recovery from first-episode psychosis in adults: systematic review and meta-analysis of long-term outcome studies. The British Journal of Psychiatry. 2017 Dec;211(6):350-8.

The correct answer is: Recovery rates stabilise after the first 2 years of illness

Question 53

Not answered

Marked out of 1.00

A 23-year-old man is diagnosed with schizophrenia. Which of the following is the most common attribute of schizophrenia?

Select one:

- ☐ Delusions
- ☐ Formal thought disorder
- ☐ Auditory hallucinations
- ☐ Loss of insight
- ☐ Body image disturbance

Your answer is incorrect.

Explanation:

Poor insight is a core attribute of schizophrenia, occurring in 58-98% of patients. Insight is an important outcome predictor, associated with treatment adherence, relapse frequency, symptom remission, psychosocial functioning, vocational attainment, and risk of violence toward self or others.

Ref: Lehrer DS, Lorenz J. Anosognosia in schizophrenia: hidden in plain sight. Innovations in clinical neuroscience. 2014 May 1;11(5-6):10.

The correct answer is: Loss of insight

Question 54

Not answered

Marked out of 1.00

What are the most common cardiac abnormalities in anorexia nervosa?

Select one:

- ☐ Tachycardia and ventricular fibrillation
- ☐ Ventricular fibrillation and Torsades des Pointes
- ☐ Tachycardia and QT interval prolongation
- ☐ Bradycardia and QT interval prolongation
- ☐ Bradycardia and Torsades des Pointes

Your answer is incorrect.

Explanation:

The most common cardiac abnormalities in anorexia nervosa are bradycardia and QT interval prolongation, which may occasionally degenerate into ventricular arrhythmias such as Torsades des Pointes or ventricular fibrillation. As these arrhythmias may be the substrate of sudden cardiac death, they require cardiac monitoring in hospital. In addition, reduced cardiac mass, with smaller volumes and decreased cardiac output, may be found. Furthermore, mitral prolapse and a mild pericardial effusion may occur, the latter due to protein deficiency and low levels of thyroid hormone.

Ref: Giovinazzo S et al. Anorexia nervosa and heart disease: a systematic review. Eating and Weight Disorders-Studies on Anorexia, Bulimia and Obesity. 2019 Apr;24(2):199-207.

The correct answer is: Bradycardia and QT interval prolongation

Question 55

Not answered

Marked out of 1.00

Which of the following is true regarding folie-a-deux?

Select one:

- ☐ It is more common in men
- ☐ Without intervention, it usually resolves spontaneously
- ☐ It is coded under Delusional disorder in ICD-11
- ☐ It often involves bizarre delusions
- ☐ It is more common in children

Your answer is incorrect.

Explanation:

ICD-10 categories F22 'Persistent delusional disorder', F23.3 'Other acute predominantly delusional psychotic disorder', and F24 'Induced delusional disorder', which is a very rare entity, have been collapsed in ICD-11 into a single diagnostic category of 'Delusional disorder'. Specifically, ICD-11 codes Induced delusional disorder under 'Delusional disorder, unspecified'. ICD-11 states that 'rarely, delusional disorder may occur at the same time (or closely associated in time) in two persons who have a strong emotional or situational link. This condition is often referred to as shared or induced delusional disorder or 'folie-a-deux'. In such cases, one person typically adopts the delusional belief of the other person, and the delusions may remit in the less dominant person when the two individuals are separated'.

There is a predominance of female sex and a high occurrence of cases in the context of a close relationship, often among members of the same family. Data about age are contrasting and do not demonstrate a consistent pattern; there is a broad range of ages. As for symptomatological features, delusions are the most common, with the highest prevalence reported for persecutory and religious delusions. Without intervention, the course is usually chronic.

Ref: Stein DJ et al. Mental, behavioral and neurodevelopmental disorders in the ICD-11: an international perspective on key changes and controversies. BMC medicine. 2020 Dec;18(1):1-24.

International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 238.

The correct answer is: It is coded under Delusional disorder in ICD-11

Question **56**

Not answered

Marked out of 1.00

Which of the following medications increases serum lithium levels?

Select one:

- ☐ Imipramine
- ☐ Indomethacin
- ☐ Sertraline
- ☐ Olanzapine
- ☐ MAOIs

Your answer is incorrect.

Explanation:

Indomethacin, or indometacin, is a nonsteroidal anti-inflammatory drug (NSAID). NSAIDs inhibit the synthesis of renal prostaglandins, thereby reducing renal blood flow and possibly increasing renal re-absorption of sodium and therefore lithium. The magnitude of the rise is unpredictable for any given patient; case reports vary from increases of around 10% to over 400%. NSAIDs can be combined with lithium but they should be prescribed regularly, **not** prn, and more frequent plasma lithium monitoring is essential.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 253.

The correct answer is: Indomethacin

Question 57

Not answered

Marked out of 1.00

Which of the following statements about lithium is true?

Select one:

- ☐ Polyuria occurs mainly with once daily dosing
- ☐ Lithium causes more weight gain than sodium valproate
- ☐ Lithium increase the risk of hypoparathyroidism
- ☐ 20% of middle aged women develop hypothyroidism
- ☐ 50% of people taking lithium develop tremors

Your answer is incorrect.

Explanation:

Most adverse effects of lithium are dose and plasma level related, including mild GI upset, fine tremor, polyuria, and polydipsia. Polyuria may occur more frequently with twice-daily dosing. Tremor, primarily of the hands, is among the most common lithium side effects, seen in approximately 25% of treated patients. Lithium increases the risk of hypothyroidism; in middle-aged women, the risk may be up to 20%. Lithium also more rarely increases the risk of **hyper**thyroidism and **hyper**parathyroidism. Lithium can also cause a metallic taste in the mouth, ankle oedema, and weight gain. Lithium causes less weight gain than valproate.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 250.

Gitlin M. Lithium side effects and toxicity: prevalence and management strategies. International journal of bipolar disorders. 2016 Dec;4(1):1-0.

The correct answer is: 20% of middle aged women develop hypothyroidism

Question **58**

Not answered

Marked out of 1.00

Which of the following is not a change seen in the sleep architecture of older people?

Select one:

- ☐ More stage 1 and 2 sleep
- ☐ Longer sleep latency
- ☐ Fewer NREM-REM sleep cycles
- ☐ More awakenings
- ☐ More slow wave sleep and REM sleep

Your answer is incorrect.

Explanation:

Advancing into the fifth decade of older age and beyond are a collection of well-characterised changes in sleep architecture.



SPMM COURSE



(c) Servier Medical Art

Advanced sleep timing (i.e. earlier bedtimes and rise times)
Longer sleep latency (i.e. longer time taken to fall asleep)
Shorter overall sleep duration
Increased sleep fragmentation (i.e. less consolidated sleep with more awakenings, arousals, or transitions to lighter sleep stages)
More fragile sleep (i.e. higher likelihood of being woken by external sensory stimuli)
Reduced amount of deeper NREM sleep (slow wave sleep)
Increased time spent in lighter NREM stages 1 and 2
Shorter and fewer NREM-REM sleep cycles
Increased time spent awake throughout the night

Changes in sleep architecture with older age

Ref: Mander et al. 2017

Ref: Mander BA et al. Sleep and human aging. Neuron. 2017 Apr 5;94(1):19-36.

The correct answer is: More slow wave sleep and REM sleep



Question 59

Not answered

Marked out of 1.00

From a resilience perspective, which of the following is a key protective factor for transgender people?

Select one:

- ☐ Dead-naming
- ☐ Misgendering
- ☐ Social isolation
- ☐ Passing/blending
- ☐ Identity affirmation

Your answer is incorrect.

Explanation:

From a resilience perspective, gender identity affirmation by others (e.g., using affirmative pronouns, avoiding 'dead-naming') serves as a key protective factor for transgender people. Behaviours such as 'misgendering' (using words, especially pronouns, that do not reflect that person's gender identity) or 'dead-naming' (referring to a transgender person by the name they used prior to transitioning, such as their birth name) are understood by proponents of gender theory to be destructive, debasing, and dehumanising. Research has demonstrated the importance of transgender people being affirmed in their gender identities by others, including during initial transitions as well as in cases of retransition (whether from binary to non-binary, the reverse, or back to a cisgender identity). Given the importance of the social environment, transgender people actively seek out and construct communities and spaces in which they receive validation and affirmation of their gender identities.

Ref: Doyle DM. Transgender Identity: Development, Management and Affirmation. Current Opinion in Psychology. 2022 Sep 15;101467.

Griffin L et al. Sex, gender and gender identity: a re-evaluation of the evidence. BJPsych Bulletin. 2021 Oct;45(5):291-9.

The correct answer is: Identity affirmation

Question 60

Not answered

Marked out of 1.00

Which of the following drugs has been found consistently in clinical trials to reduce the negative symptoms of schizophrenia?

Select one:

- ☐ Zotepine
- ☐ Risperidone
- ☐ Chlorpromazine
- ☐ Perazine
- ☐ None of the listed treatments

Your answer is incorrect.

Explanation:

Antipsychotics have limited clinical effectiveness for negative symptoms. Amisulpride is the only medication approved for schizophrenia with prominent negative symptoms. More recently cariprazine, a D3-preferring partial agonist, has been approved as second line restricted agent for predominant negative symptoms (superior to placebo and risperidone but not aripiprazole). Clozapine appears superior to other antipsychotics in managing negative symptoms.

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

Earley W, et al. Efficacy of cariprazine on negative symptoms in patients with acute schizophrenia: a post hoc analysis of pooled data. Schizophrenia research. 2019 Feb 1;204:282-8.

The correct answer is: None of the listed treatments

Question **61**

Not answered

Marked out of 1.00

What is the risk of developing schizophrenia for a child born to one parent with schizophrenia?

Select one:

- ☐ 2-5%
- ☐ 6-9%
- ☐ 40-46%
- ☐ 12-15%
- ☐ 22-25%

Your answer is incorrect.

Explanation:

The likelihood of a person having schizophrenia is correlated with the closeness of the relationship to an affected relative. Having a first-degree relative (e.g. one parent or one sibling) with schizophrenia increases a person's risk to 12-15%. The risk jumps to 40% if both parents have schizophrenia. In the case of monozygotic twins who have identical genetic endowment, there is an approximately 46% concordance rate for schizophrenia.

Schizophrenia liability based on affected relatives

Family member(s) affected	Risk of schizophrenia (approximate) (%)
Identical twin	46
One sibling/fraternal twin	12-15
Both parents	40
One parent	12-15
One grandparent	6
No relatives affected	0.5-1

Adapted from Oxford Handbook of Psychiatry, 2019. p. 191.



Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 360.

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 191.

The correct answer is: 12-15%



Question 62

Not answered

Marked out of 1.00

Hoover test is a physical examination manouver done to identify the possible diagnosis of

Select one:

- ☐ Hypochondriasis
- ☐ Munchausen syndrome
- ☐ Major depressive disorder
- ☐ Somatisation disorder
- ☐ Dissociative neurological symptom disorder

Your answer is incorrect.

Explanation:

Hoover test has been used to differentiate between organic and functional hemiparesis. In conversion disorder (also known as functional neurological symptom disorder in DSM-5 or dissociative neurological symptom disorder in ICD-11), the patient is unable to raise the affected limb from the couch but is able to raise the unaffected limb against resistance, with demonstrable pressing down of the heel on the 'affected' side.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 868.

The correct answer is: Dissociative neurological symptom disorder

Question 63

Not answered

Marked out of 1.00

A 33 year old woman has had two previous episodes of documented hypomania and one episode of depression. She is interested in treatment that can prevent relapses. Which of the following is least likely to be beneficial?

Select one:

- ☐ Psychoanalytic psychotherapy
- ☐ Lithium
- ☐ Interpersonal and social rhythms therapy
- ☐ CBT
- ☐ Valproate

Your answer is incorrect.

Explanation:

NICE recommends offering lithium as first-line, long-term pharmacological treatment for bipolar disorder. If lithium is insufficiently effective, consider adding valproate. If lithium is poorly tolerated, consider valproate or olanzapine instead, or if it has been effective during an episode of mania or bipolar depression, quetiapine. Evidence-based psychotherapeutic treatments in bipolar disorder include family therapy, cognitive behavioural therapy, psychoeducation, and interpersonal and social rhythms therapy.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 297, 303.

The correct answer is: Psychoanalytic psychotherapy

Question **64**

Not answered

Marked out of 1.00

What would be the probable diagnosis for a person experiencing multiple worries, tension headaches, difficulty concentrating, and feelings of being on edge all the time?

Select one:

- ☐ Avoidant personality disorder
- ☐ Social anxiety disorder
- ☐ Agoraphobia
- ☐ Hypochondriasis
- ☐ Generalised anxiety disorder

Your answer is incorrect.

Explanation:

Generalised anxiety disorder is characterised by marked symptoms of anxiety that persist for at least several months, for more days than not, manifested by either general apprehension (i.e. 'free-floating anxiety') or excessive worry focused on multiple everyday events, most often concerning family, health, finances, and school or work, together with additional symptoms such as muscular tension or motor restlessness, sympathetic autonomic overactivity, subjective experience of nervousness, difficulty maintaining concentration, irritability, or sleep disturbance.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed,; ICD-11; World Health Organization, 2020)

The correct answer is: Generalised anxiety disorder

Question 65

Not answered

Marked out of 1.00

A 32-year-old woman presents with anxiety and preoccupation that her nose is grossly misshapen. She is not convinced otherwise, despite reassurances from her friends and family that she is attractive and 'normal'. Which of the following interventions has evidence-based support in this situation?

Select one:

- ☐ Pimozide
- ☐ Olanzapine
- ☐ Fluoxetine
- ☐ Clonazepam
- ☐ Plastic surgery

Your answer is incorrect.

Explanation:

Clinical guidelines recommend SSRIs for the pharmacological treatment of body dysmorphic disorder. Most evidence comes from open trials, with only four RCTs conducted to date. RCTs have shown evidence for the efficacy of fluoxetine and escitalopram. To date, there has been one small open trial and one RCT that evaluated pimozide and olanzapine augmentation of fluoxetine, respectively. These studies did not find beneficial effects of augmentation with either medication. Plastic surgery to the affected part is generally not indicated. Even 'successful' surgery risks being followed by a new preoccupation or a focus on surgical scarring.

Ref: Krebs G et al. Recent advances in understanding and managing body dysmorphic disorder. Evidence-Based Mental Health. 2017 Aug 1;20(3):71-5.

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 872.

The correct answer is: Fluoxetine

Question 66

Not answered

Marked out of 1.00

Based on currently available evidence, which of the following strategies may be used in the treatment of negative symptoms in schizophrenia?

Select one:

- ☐ Switch from first-generation to second-generation antipsychotic
- ☐ Antidepressant add-on to antipsychotic treatment
- ☐ Social skills training
- ☐ Supported employment and supported housing
- ☐ All of the listed options

Your answer is incorrect.

Explanation:

Based on currently available evidence, the European Psychiatric Association (EPA) has issued updated recommendations for the treatment of undifferentiated negative symptoms (including both primary and secondary negative symptoms) in schizophrenia. Although it is difficult to formulate an evidence-based recommendation for the choice of an antipsychotic, a switch to a second-generation antipsychotic should be considered for patients who are treated with a first-generation antipsychotic. Antidepressant add-on to antipsychotic treatment is an option. Social skills training is recommended, as well as cognitive remediation for patients who also show cognitive impairment. Exercise interventions also have shown promise. Finally, access to treatment and to rehabilitation interventions (such as supported employment and supported housing) should be ensured for patients with negative symptoms.

Ref: Galderisi S et al. EPA guidance on treatment of negative symptoms in schizophrenia. European Psychiatry. 2021;64(1).

The correct answer is: All of the listed options

Question **67**

Not answered

Marked out of 1.00

Which of the following will help with recovery from PTSD?

Select one:

- ☐ Avoidance
- ☐ Social support
- ☐ Not talking about the experience
- ☐ Litigation
- ☐ Thought suppression

Your answer is incorrect.

Explanation:

Outcome in PTSD depends on initial symptom severity. Recovery will be helped by good social support; lack of negative responses from others; absence of 'maladaptive' coping mechanisms (e.g. avoidance, denial, 'safety behaviours', not talking about the experience, thought suppression, rumination); and no further traumatic life events (secondary problems such as physical health, acquired disability, disfigurement, disrupted relationships, financial worries, and litigation).

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 872.

The correct answer is: Social support

Question **68**

Not answered

Marked out of 1.00

Which medication is most likely to exacerbate psoriasis?

Select one:

- ☐ Sodium valproate
- ☐ Clozapine
- ☐ Chlorpromazine
- ☐ Carbamazepine
- ☐ Lithium

Your answer is incorrect.

Explanation:

Some skin conditions such as psoriasis and acne can be aggravated by lithium therapy.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 250.

The correct answer is: Lithium



Question 69

Not answered

Marked out of 1.00

Which of the following peri-traumatic factors are predictive of the future development of PTSD?

Select one:

- ☐ Dissociation
- ☐ Autonomic arousal
- ☐ Experiencing a panic attack
- ☐ Anterograde amnesia
- ☐ Crying

Your answer is incorrect.

Explanation:

Cumulative exposure to potentially traumatic experiences, trauma severity, and being trapped during an earthquake are peri-traumatic factors associated with PTSD. Dissociation, being injured, witnessing injury or death during trauma, and bereavement (i.e., losing family members or close friends as a result of the traumatic experience) are also risk factors for PTSD. The subjective response to the trauma is predictive, with acute dissociative reactions and catastrophic appraisals about the outcome of the event being strongly associated with later PTSD severity. Of note, despite the relationship between peri-traumatic dissociation and later PTSD, many people who develop PTSD do not display dissociative responses in the acute phase after trauma.

Ref: Tortella-Feliu M et al. Risk factors for posttraumatic stress disorder: An umbrella review of systematic reviews and meta-analyses. Neuroscience & Biobehavioral Reviews. 2019 Dec 1;107:154-65.

Bryant RA. Post-traumatic stress disorder: a state-of-the-art review of evidence and challenges. World psychiatry. 2019 Oct;18(3):259-69.

The correct answer is: Dissociation

Question **70**

Not answered

Marked out of 1.00

Which one of the following is employed as a screening tool in clinical psychiatry?

Select one:

- ☐ Young Mania Rating Scale
- ☐ Hamilton Depression Rating Scale
- ☐ Edinburgh Postnatal Depression Scale
- ☐ Montgomery Asberg Depression Rating Scale
- ☐ Yale-Brown Obsessive Compulsive Scale

Your answer is incorrect.

Explanation:

The Edinburgh Postnatal Depression Scale is a 10-item tool developed for screening postpartum women for depression in outpatient clinics, home visits, or during postpartum appointments.

Ref: Smith-Nielsen J et al. Validation of the Edinburgh Postnatal Depression Scale against both DSM-5 and ICD-10 diagnostic criteria for depression. BMC psychiatry. 2018 Dec;18(1):1-2.

The correct answer is: Edinburgh Postnatal Depression Scale

Question **71**

Not answered

Marked out of 1.00

According to DSM-5 criteria, which of the following features must be present for a diagnosis of neuroleptic malignant syndrome to be made?

Select one:

- ☐ Hypertension and confusion
- ☐ Rigidity and pyrexia
- ☐ Rigidity and leukocytosis
- ☐ High creatinine kinase and rigidity
- ☐ Pyrexia and leukocytosis

Your answer is incorrect.

Explanation:

medicine
usually
acid include
aspiration
Heart
low oxygen change
failure
NMS
body
pressure
treated might
damage within
lungs changed
symptoms
dosage

(c) Servier Medical Art

002986

Cardinal features:

- **Exposure to a dopamine antagonist** within 72 hours prior to symptom development
- **Hyperthermia** (>38C orally on at least two occasions)
- **Generalised rigidity** ("lead pipe" in most severe form)

Other possible signs:

- **Neurological:** tremor, sialorrhea, akinesia, dystonia, trismus, dysarthria, dysphagia, rhabdomyolysis
- **Changes in mental status:** delirium, altered consciousness (ranging from stupor to coma), or catatonic stupor
- **Autonomic activation/instability:** tachycardia, diaphoresis, blood pressure elevation or fluctuation, urinary incontinence
- **Tachypnea/respiratory distress**
- **CK elevation** (at least four times above upper normal limit)
- **Other possible lab findings:** leukocytosis, metabolic acidosis, hypoxia, decreased serum iron concentrations, elevations in serum muscle enzymes and catecholamines

Neuroleptic Malignant Syndrome

SPMM COURSE



Ref: American Psychiatric Association. Diagnostic and statistical manual of mental disorders (DSM-5®). American Psychiatric Pub; 2013 May 22.

The correct answer is: Rigidity and pyrexia

Question **72**

Not answered

Marked out of 1.00

A young man in your clinic complains of extrapyramidal side effects. Which of the following would you use to measure this?

Select one:

- ☐ AIMS (Abnormal Involuntary Movements Scale)
- ☐ Simpson-Angus Scale
- ☐ PANSS (Positive and Negative Symptom Scale)
- ☐ Young Mania Rating Scale
- ☐ Morgan-Russell Scale

Your answer is incorrect.

Explanation:

The Simpson-Angus EPS Rating Scale is a widely-used, 10-item rating scale that measures drug-induced parkinsonism (extrapyramidal side effects) including bradykinesia and tremor.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 110.

The correct answer is: Simpson-Angus Scale



Question **73**

Not answered

Marked out of 1.00

Mr. X is a 36-year-old man who has a long-standing diagnosis of schizophrenia. He remains psychotic despite multiple medication trials including haloperidol, olanzapine and risperidone. You would like to consider him for a trial of clozapine. Which of the following is the most important investigation(s) to be ordered before starting clozapine?

Select one:

- ☐ Urea and electrolytes
- ☐ ECG
- ☐ Thyroid functioning
- ☐ White blood cell count
- ☐ Liver function tests

Your answer is incorrect.

Explanation:

CLOZAPINE INITIATION

Assess eligibility

- A lack of adequate response despite the use of adequate doses of at least two different antipsychotic agents, including an atypical, prescribed for adequate duration

Assess safety

- Assess for pre-existing medical condition, smoking and other prescribed drugs
- Measure BMI, waist circumference, postural BP stability, pulse rate
- Request CBC with differential, renal and hepatic function, ECG, fasting glucose and lipids
- Before and after dose (2-6hrs) vital sign monitoring for 2 weeks
- Repeat cardiac enzymes/CRP weekly for 4 weeks

Psychoeducation

- Explain treatment resistance
- Present the possibility of side effects and mitigations available
- Explain the monitoring requirements
- Stress the need for adherence
- Discuss the lifestyle changes needed

Titration plan

- Start at 12.5 mg at night; increase by 25mg/day with more dose at night preferably
- At 200mg/d dose, taper off previous antipsychotic
- Titrate based on clinical response and side effects
- Target dose 300-450mg/d; further increase <100mg/week, max 900mg/d

SPMM COURSE

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

The correct answer is: White blood cell count

Question **74**

Not answered

Marked out of 1.00

Which of the following drugs is a recognised cause of depression?

Select one:

- ☐ Citalopram
- ☐ Midazolam
- ☐ Risperidone
- ☐ Quetiapine
- ☐ Clonidine

Your answer is incorrect.

Explanation:

Clonidine is an adrenergic alpha2 agonist. Its main side effects are sedation, postural hypotension, and depression.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 588.

The correct answer is: Clonidine



Question 75

Not answered

Marked out of 1.00

Which of the following psychotropics significantly interferes with oral contraceptives?

Select one:

- ☐ Valproate
- ☐ Lamotrigine
- ☐ Risperidone
- ☐ Olanzapine
- ☐ Carbamazepine

Your answer is incorrect.

Explanation:

Carbamazepine is a potent inducer of hepatic cytochrome enzymes and is metabolised by CYP3A4. Plasma levels of most antidepressants, most antipsychotics, benzodiazepines, warfarin, zolpidem, some cholinesterase inhibitors, methadone, thyroxine, theophylline, **oestrogens** and other steroids may be reduced by carbamazepine, resulting in treatment failure. Patients requiring **contraception** should either receive a **preparation containing not less than 50 micrograms oestrogen** or use a **non-hormonal method**.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 266.

The correct answer is: Carbamazepine

Question **76**

Not answered

Marked out of 1.00

Depression rating scales may overlap with negative symptoms of schizophrenia. Which of the following rating scales is specific for measuring symptoms of depression in those with schizophrenia?

Select one:

- ☐ Montgomery-Asberg Depression Rating Scale
- ☐ Zung Depression Scale
- ☐ Schedule for the deficit syndrome
- ☐ Calgary Depression Scale
- ☐ Centre for epidemiological studies depression scale (CES-D)

Your answer is incorrect.

Explanation:

The Calgary Depression Scale for Schizophrenia is a 9-item clinician-rated outcome measure that assesses level of depression in people with schizophrenia. It is better able to differentiate depression features from negative symptoms than other standard depression rating scales.

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

The correct answer is: Calgary Depression Scale

Question 77

Not answered

Marked out of 1.00

A patient with bipolar disorder is already maintained on quetiapine but experiences a manic episode. Which of the following drugs would you use to treat the current episode of mania?

Select one:

- ☐ Carbamazepine
- ☐ Lithium
- ☐ Aripiprazole
- ☐ Lamotrigine
- ☐ Olanzapine

Your answer is incorrect.

Explanation:

BAP guidelines state that if acute mania occurs while a patient with bipolar disorder is taking optimised doses of a first-line maintenance medication (such as quetiapine), the combination of lithium or valproate with a dopamine antagonist/partial agonist should be considered.

Ref: Goodwin GM et al. Evidence-based guidelines for treating bipolar disorder: revised third edition recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2016 Jun;30(6):495-553.

The correct answer is: Lithium

Question **78**

Not answered

Marked out of 1.00

The risk of dying by completed suicide in the 2 years after an act of self-harm is

Select one:

- ☐ 1%
- ☐ 5%
- ☐ 10%
- ☐ 3%
- ☐ 0.2%

Your answer is incorrect.

Explanation:

Amongst patients who perform an act of self-harm, roughly 1% will die by completed suicide in the 24 months after the initial act, with the risk highest in the weeks following the original act. This represents a mortality by suicide 50-100 times that of the general population.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 848.

The correct answer is: 1%



Question 79

Not answered

Marked out of 1.00

Which of the following is associated with an increase in impulsivity in patients with borderline personality disorder?

Select one:

- ☐ Mentalisation therapy
- ☐ Benzodiazepine therapy
- ☐ Mood stabiliser therapy
- ☐ Partial hospitalisation
- ☐ Mindfulness therapy

Your answer is incorrect.

Explanation:

Benzodiazepines are strongly discouraged in treating borderline personality disorder due to risk of worsening impulsivity and suicidality. Borderline patients may also be at increased risk for benzodiazepine use disorders, in an effort to self-medicate chronic, refractory affective symptoms by fostering dissociative symptomatology.

Ref: Ripoll LH. Psychopharmacologic treatment of borderline personality disorder. Dialogues in clinical neuroscience. 2013 Jun;15(2):213.

The correct answer is: Benzodiazepine therapy

Question 80

Not answered

Marked out of 1.00

Which of the following drugs is contraindicated during breastfeeding?

Select one:

- ☐ Olanzapine
- ☐ Clozapine
- ☐ Risperidone
- ☐ Aripiprazole
- ☐ Sulpride

Your answer is incorrect.

Explanation:

Women with a mental health diagnosis should be encouraged to breastfeed UNLESS they are taking carbamazepine, clozapine, or lithium. Lithium levels can be high in breast milk, with a risk of toxicity for the baby. Clozapine is associated with a risk of neonatal seizures and blood dyscrasias.

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

McAllister-Williams RH et al. British Association for Psychopharmacology consensus guidance on the use of psychotropic medication preconception, in pregnancy and postpartum 2017. Journal of Psychopharmacology. 2017 May;31(5):519-52.

National Institute for Health and Clinical Excellence. Antenatal and postnatal mental health: clinical management and service guide. Updated edition 2020.

The correct answer is: Clozapine

Question **81**

Not answered

Marked out of 1.00

Which of the following drugs would be the most effective treatment for a 38-year-old man with acute mania?

Select one:

- ☐ Sodium valproate
- ☐ Lamotrigine
- ☐ Topiramate
- ☐ Carbamazepine
- ☐ Gabapentin

Your answer is incorrect.

Explanation:

BAP guidelines recommend that for acute mania, a dopamine antagonist should be considered for rapid antimanic effect (haloperidol, olanzapine, quetiapine, or risperidone). Valproate is an alternative (less risk of adverse motor reactions) but should not be used for women of child-bearing potential due to the risk of teratogenesis and impaired foetal intellectual development.

Ref: Goodwin GM et al. Evidence-based guidelines for treating bipolar disorder: revised third edition recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2016 Jun;30(6):495-553.

The correct answer is: Sodium valproate

Question 82

Not answered

Marked out of 1.00

A 32-year-old woman with a diagnosis of anorexia nervosa is transferred from an eating disorder unit to a medical emergency unit after losing consciousness. On admission, she appears extremely emaciated, with low blood pressure and pulmonary oedema. Which of the following complications of anorexia is the most likely explanation for her presentation?

Select one:

- ☐ Sinus tachycardia
- ☐ Myocarditis
- ☐ First degree heart block
- ☐ Cardiomyopathy
- ☐ Pulmonary embolism

Your answer is incorrect.

Explanation:

Cardiomyopathy has been reported in patients with anorexia nervosa, possibly due to diminished protein synthesis, malnutrition, direct cardiotoxicity from emetics, and refeeding syndrome.

Ref: Sardar MR et al. Cardiovascular impact of eating disorders in adults: a single center experience and literature review. Heart views: the official journal of the Gulf Heart Association. 2015 Jul;16(3):88.

The correct answer is: Cardiomyopathy

Question **83**

Not answered

Marked out of 1.00

A 33-year-old man with history of schizophrenia has moderately severe renal failure. Which of the following drugs undergoes significant renal excretion?

Select one:

- ☐ Olanzapine
- ☐ Aripiprazole
- ☐ Chlorpromazine
- ☐ Risperidone
- ☐ Amisulpride

Your answer is incorrect.

Explanation:

Amisulpride is primarily renally excreted. 50% is excreted unchanged in the urine. There is limited experience with amisulpride in renal disease and it is best avoided in established renal failure.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 738.

The correct answer is: Amisulpride

Question **84**

Not answered

Marked out of 1.00

You are seeing a patient with chronic schizophrenia who has always complained of dry mouth. Which of the following antipsychotics is least likely to cause this adverse effect?

Select one:

- ☐ Quetiapine
- ☐ Trifluoperazine
- ☐ Amisulpride
- ☐ Chlorpromazine
- ☐ Olanzapine

Your answer is incorrect.

Explanation:

Amisulpride is not associated with dry mouth and may even be considered for dry mouth treatment.

Ref: Godoy T et al. Atypical antipsychotics—effects of amisulpride on salivary secretion and on clozapine-induced sialorrhea. Oral diseases. 2012 Oct;18(7):680-91.

The correct answer is: Amisulpride



Question 85

Not answered

Marked out of 1.00

Affective dysregulation is a principal psychopathology of which of the following disorders?

Select one:

- ☐ Dissociative fugue
- ☐ Histrionic personality disorder
- ☐ Dysthymia
- ☐ Acute stress disorder
- ☐ Borderline personality disorder

Your answer is incorrect.

Explanation:

Patients with borderline personality disorder are characterised by unstable affect, mood, behaviour, object relations, and self-image. Clinical features include: unstable self-image; unstable interpersonal relationships; impulsive behaviour in areas such as personal safety and substance misuse; self-harm, suicidal threats or gestures; affective instability due to marked reactivity of mood; episodes of micro-psychosis; chronic feelings of emptiness; frantic efforts to avoid real or imagined abandonment; inappropriate, intense anger.

Ref: American Psychiatric Association. Diagnostic and statistical manual of mental disorders (DSM-5®). American Psychiatric Pub; 2013 May 22.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 564.

The correct answer is: Borderline personality disorder

Question 86

Not answered

Marked out of 1.00

In the 1970s, Taylor and Abrams challenged the limited recognition of catatonia only as schizophrenia, reporting catatonia to be more common among manic and depressed patients than among those with schizophrenia. Which of the following features is most traditionally associated with catatonic schizophrenia?

Select one:

- ☐ Negativism
- ☐ Mannerisms
- ☐ Automatisms
- ☐ Stereotypies
- ☐ Echopraxia

Your answer is incorrect.

Explanation:

Taylor and Abrams, in 4 publications between 1973 and 1979, reported catatonia to be more common among manic and depressed patients than among those with schizophrenia, challenging the limited recognition of catatonia only as schizophrenia. They re-established that most catatonic patients have a mood disorder, particularly mania, and that 20% of patients with mania exhibit catatonic features. About 10-15% of patients with catatonia meet the criteria for schizophrenia when the diagnosis requires no past or present episodes of mood disorder. Catalepsy, mannerisms, posturing, and mutism are the features traditionally associated with catatonic schizophrenia.

Ref: Fink M et al. Catatonia is not schizophrenia: Kraepelin's error and the need to recognize catatonia as an independent syndrome in medical nomenclature. Schizophrenia bulletin. 2010 Mar 1;36(2):314-20.

Taylor MA, Fink M. Catatonia in psychiatric classification: a home of its own. American Journal of Psychiatry. 2003 Jul 1;160(7):1233-41.

Rasmussen SA et al. Catatonia: our current understanding of its diagnosis, treatment and pathophysiology. World journal of psychiatry. 2016 Dec 22;6(4):391.

The correct answer is: Mannerisms

Question **87**

Not answered

Marked out of 1.00

The proportion of patients diagnosed with schizophrenia that experience recurrent relapse and continued disability is

Select one:

- ☐ About 5%
- ☐ About 20%
- ☐ About 50%
- ☐ About 90%
- ☐ About 75%

Your answer is incorrect.

Explanation:

Most patients recover from initial acute phase of schizophrenia, though many don't recover fully and continue to have residual symptoms. There is considerable variation in long-term outcome between patients. $\frac{3}{4}$ (75%) experience recurrent relapse and continued disability. Recurrent episodes/relapses are often related to stress, adversity, social isolation, and poor adherence to treatment. Though the prognosis of schizophrenia is often thought of pessimistically, over half have moderately good long-term global outcomes.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the treatment and management of psychosis and schizophrenia in adults. Updated edition 2014.

The correct answer is: About 75%

Question 88

Not answered

Marked out of 1.00

Which one of the following best reflects the evidence on the adverse effects of electroconvulsive therapy (ECT) when used in the treatment of moderate to severe depression?

Select one:

- ☐ Simulated ECT causes the same degree of memory loss as ECT immediately after treatment
- ☐ Bilateral ECT is more likely to cause cognitive impairment than unilateral ECT
- ☐ ECT is more likely to cause profound cognitive impairment six months after treatment than simulated ECT
- ☐ Low dose ECT is more likely to cause transient memory loss than high dose ECT
- ☐ ECT causes no direct effects on memory in non-depressed individuals

Your answer is incorrect.

Explanation:

The Royal College of Psychiatrists' statement on ECT (2017) notes the following:

- Cognitive effects are the main limitation to the wider use of ECT, particularly the occasional acute confusion shortly after the treatment, retrograde amnesia, and some losses in autobiographical (personal) memory longer term
- Bilateral ECT is more effective than unilateral ECT but may cause more cognitive impairment
- Right unilateral electrode placement, compared with the traditional bi-temporal placement, may minimise episodic and autobiographical memory deficits
- With unilateral ECT, a higher stimulus dose is associated with greater efficacy, but also increased cognitive impairment compared with a lower stimulus dose
- Continuation ECT is not associated with adverse memory outcomes

Ref: Royal College of Psychiatrists. Statement on electroconvulsive therapy (ECT). 2017

The correct answer is: Bilateral ECT is more likely to cause cognitive impairment than unilateral ECT



© 2023 SPMM Course Limited, UK.

[Refund Policy](#) | [Terms & Conditions](#)

